INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Dose-dense AC → weekly paclitaxel

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
14 days
Intervals differ by treatment phase

Treatment stages
2
Explained in the treatment reference

Regimen source
NCCP 00485
Version 6

Four AC treatments every 14 days, then 12 weekly paclitaxel doses starting 14 days after the last AC. About 20 weeks without delays.

Treatment stages

01 **AC chemotherapy**
4 cycles, every 14 days

02 **Weekly paclitaxel**
12 weekly doses

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

		PAGE
01	Understanding your treatment	2
02	Your schedule, tests and appointments	6
03	Symptoms and supportive care	12
04	Your record and contact card	18
05	Sources and further information	22

My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Report new breathlessness, jaundice, marked bleeding, severe diarrhoea, inability to drink, chest symptoms or a severe rash promptly.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

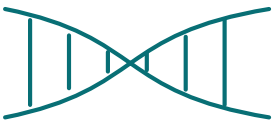
This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Your medicines and what they do

Reduce recurrence risk as part of the agreed early-breast treatment plan.

ANTHRACYCLINE CHEMOTHERAPY

Doxorubicin



Interferes with deoxyribonucleic acid (the cell's genetic material) (DNA) handling

Doxorubicin interferes with enzymes and DNA processes needed for cell division. It can also affect heart muscle, so the team considers previous exposure and cardiac risk.

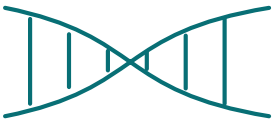
DNA handling disrupted → Cell damage → Division reduced

What to watch for

Report pain or burning at the infusion site immediately. New breathlessness or chest symptoms need prompt review. Red urine for a short time can be a drug-colour effect; pain, clots or persistent bleeding still need advice.

ALKYLATING CHEMOTHERAPY

Cyclophosphamide



Damages DNA needed for growth

Cyclophosphamide is activated in the body and damages DNA. This helps stop dividing cancer cells but can also affect blood-cell production and reproductive tissues.

Drug activated → DNA damage → Cell division affected

What to watch for

Use the prescribed anti-sickness and hydration plan. Report fever, painful urination or blood in the urine.

TAXANE CHEMOTHERAPY

Paclitaxel

Taxol (conventional paclitaxel)



Locks the cell division machinery

Cells use tiny structures called microtubules to separate their chromosomes when dividing. Paclitaxel stabilises these structures so they cannot rearrange normally, interrupting cell division. It can also affect nerves and other healthy tissues.

Microtubules form → Structures remain locked → Division is interrupted

What to watch for

Report tingling or numbness before the next weekly dose, especially if it affects walking or hand use. Tell the nurse immediately about flushing, chest or throat tightness, itching, dizziness or breathing changes during infusion.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Why this treatment?

This anthracycline–taxane sequence is an option for selected high-risk early breast cancer. Dose-dense AC needs growth-factor support; the later weekly paclitaxel phase has different premedicines and toxicity priorities.

Your treatment setting

Adjuvant or neoadjuvant high-risk node-negative or node-positive breast cancer

Your exact schedule

Four AC treatments (doxorubicin 60 mg/m² and cyclophosphamide 600 mg/m²) 14 days apart, followed 14 days after the last AC by 12 weekly paclitaxel 80 mg/m² doses, subject to recovery. The planned sequence spans about 20 weeks.

Alternatives

Discuss biomarker-directed, endocrine, chemotherapy and supportive-care options appropriate to this setting, including the option not to proceed.

Why this treatment may be chosen

Discuss how this evidence and the available alternatives apply to your own situation.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Treatment and review

Treatment stages

01 **AC chemotherapy**
4 cycles, every 14 days

02 **Weekly paclitaxel**
12 weekly doses

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

4 CYCLES, EVERY 14 DAYS
AC chemotherapy

4 cycles, every 14 days

Growth-factor support is required for each dose-dense AC cycle. The anti-sickness prescription belongs to this phase.

12 WEEKLY DOSES
Weekly paclitaxel

12 weekly doses

Premedicines and anti-sickness support change. Bloods and numbness are checked before each weekly dose.

Your team confirms the active medicines, treatment days, dose changes and next review.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Relative milestones, not equal time intervals. Your team books tests and reviews results; this map does not confirm completion.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00485 v6 · official regimen · ESMO 2024 early breast cancer guideline · treatment and follow-up · NICE NG101 · bone health and follow-up

THE WHOLE TREATMENT JOURNEY

Dose-dense AC → weekly paclitaxel: treatment and checks

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First treatment	During treatment	Change / completion	Follow-up
Treatment plan NCCP 00485	Eligibility and cardiac risk	AC q14 days ×4	Paclitaxel weekly ×12	Agree local/ endocrine treatment	Early-breast follow-up
 **Blood / organ function** NCCP 00485	full blood count (FBC), renal, liver baseline	Review results before AC	FBC, kidney and liver tests before each cycle; team confirms checks before weekly paclitaxel doses	Confirm recovery before restart / transition	Investigate persistent problems
Heart / nerves NCCP 00485	 echocardiogram (heart ultrasound) (ECHO)/ multigated acquisition (heart pumping) scan (MUGA) if >65 or indicated; doxorubicin left ventricular ejection fraction (heart pumping measurement) (LVEF) >50%	Review symptoms / anthracycline exposure	Cardiac review as indicated; taxane neuropathy	Review cumulative dose and symptoms	Report new cardiac symptoms
Support changes NCCP 00485	Agree growth-factor product / days	granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) required during dose-dense AC	Switch to weekly-paclitaxel premedicines	Do not carry AC support across automatically	Review continuing support needs
 **Cancer review** EARLY	Confirm setting	Agree review plan	Follow the early-breast plan	Next treatment / completion	Agreed surveillance

What each check is for

1 Before treatment

Before the first dose

Adjuvant or neoadjuvant high-risk node-negative or node-positive breast cancer. FBC, kidney and liver tests at baseline and before each cycle, with the unit confirming weekly paclitaxel test requirements. Review current illness and past dose delays separately.

Ask / act: Confirm eligibility, test results and medicine list.

2 Other checks

At the specified review points

Review lifetime anthracycline exposure and cardiac risk. Echo/MUGA is indicated above age 65 or when clinically indicated; the source requires LVEF above 50% for doxorubicin. Reassess cardiac symptoms and neuropathy as indicated; no trastuzumab is included.

Ask / act: Confirm who arranges and reviews each check.

3 Support and practical plan

At each treatment or phase change

G-CSF is required for every dose-dense AC cycle. Prescribe product, dose and timing separately under the approved local plan. First paclitaxel dose: dexamethasone 8 mg intravenous (IV), chlorphenamine 10 mg IV and famotidine 20 mg IV, each 30 minutes before treatment (NCCP suggested table).

Ask / act: Choose the matching support plan; confirm active medicines.

4 Response and follow-up

Agreed review points

After early-breast treatment, breast imaging and clinical review follow the agreed plan. Routine computed tomography (CT)/positron emission tomography (PET) scans or blood tumour markers are not used to search for distant recurrence in someone without symptoms or clinical concerns. Keep the agreed follow-up and report persistent or new symptoms between visits.

Ask / act: Review symptoms, results and patient priorities.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your treatment and access plan

STEP 1

Before treatment

FBC, kidney and liver tests at baseline and before each cycle, with the unit confirming weekly paclitaxel test requirements. Review current illness and past dose delays separately.

STEP 2

Plan venous access

A cannula, peripherally inserted central catheter (PICC) or implanted port may be used. A port is not automatically required; insertion and removal are separate procedures with individual instructions.



STEP 3

On the treatment day

Confirm the medicines due that day, any premedicines and the reviewed blood results. Tell staff immediately about pain, burning, rash or breathlessness.

STEP 4

After the visit

No routine take-home infusion pump is required for this regimen. Keep line-care, urgent-contact and next-review instructions.

Visit length

AC and weekly paclitaxel have different visit and supportive-medicine plans. The unit confirms the complete appointment duration.

Line care

A PICC or port has its own care plan. Report fever, redness, swelling, pain, leakage or line damage; do not remove or adjust it yourself.

Treatment changes

A delayed, omitted or changed dose needs an updated plan from the treating team. Do not move doses or restart on your own.

Support for this visit

G-CSF is required for every dose-dense AC cycle. Prescribe product, dose and timing separately under the approved local plan. First paclitaxel dose: dexamethasone 8 mg IV, chlorphenamine 10 mg IV and famotidine 20 mg IV, each 30 minutes before treatment (NCCP suggested table).

Checks across treatment

Your team arranges tests and reviews the results before treatment decisions.

Blood and organ checks

Other treatment-specific checks

Review lifetime anthracycline exposure and cardiac risk. Echo/MUGA is indicated above age 65 or when clinically indicated; the source requires LVEF above 50% for doxorubicin. Reassess cardiac symptoms and neuropathy as indicated; no trastuzumab is included.

Cancer review and follow-up

After early-breast treatment, breast imaging and clinical review follow the agreed plan. Routine CT/PET scans or blood tumour markers are not used to search for distant recurrence in someone without symptoms or clinical concerns.

Bone health

Bone-health assessment follows endocrine therapy, ovarian suppression, menopause and other fracture risks. It has its own monitoring plan.

The wider treatment plan

For neoadjuvant treatment, agree response review and surgery planning. After surgery, confirm radiotherapy and endocrine treatment where appropriate; their schedules and checks remain separate.

Medicines review

Check new prescriptions, over-the-counter medicines and supplements with the oncology pharmacist before taking them.

After treatment

Keep the agreed follow-up and report persistent or new symptoms between visits.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Phase 1 · AC every 14 days

Day 1 → Day 14: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Four AC treatments 14 days apart, followed 14 days after the last AC by 12 weekly paclitaxel doses, subject to recovery and review.

Treatment or check	Timing in this view
Planned treatment	 **Day 1** Treatment
Nausea / appetite	 **Days 1–5** Can occur
Infection / low counts	 **Days 1–14** Possible at any point
Fatigue	 **Days 1–14** May build over treatment

Timing varies: Symptoms can occur on any day, including days without treatment. The bands are orientation, not predicted symptom-free periods.

Treatment-day changes: A delayed, omitted or changed dose needs an updated plan from the treating team. Do not move doses or restart on your own.

> **Why these bands stay broad:** Planned treatment days are distinct from broad possible symptom patterns. Symptoms can occur outside these bands. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00485 v6 · official regimen ↗

> **There are no guaranteed "safe days".** Fever, shivers, sudden illness, new breathing problems or severe symptoms need prompt advice on any day, including a treatment break.

Phase 2 · weekly paclitaxel

Day 1 → Day 7: a general pattern

Paclitaxel is repeated weekly for 12 doses.

Treatment or check	Timing in this view
Paclitaxel	 **Day 1** IV
Aches / fatigue	 **Days 1–7** Timing varies
Numbness / tingling	 **Days 1–7** Can accumulate
Bloods before next dose	 **Day 7** Review

Timing varies: Symptoms can occur on any day, including days without treatment. The bands are orientation, not predicted symptom-free periods.

Treatment-day changes: A delayed, omitted or changed dose needs an updated plan from the treating team. Do not move doses or restart on your own.

> **Why these bands stay broad:** Planned treatment days are distinct from broad possible symptom patterns. Symptoms can occur outside these bands. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00485 v6 · official regimen ↗

Between-treatment notes

Day 1–14: Reference cycle

Four AC treatments (doxorubicin 60 mg/m² and cyclophosphamide 600 mg/m²) 14 days apart, followed 14 days after the last AC by 12 weekly paclitaxel 80 mg/m² doses, subject to recovery. The planned sequence spans about 20 weeks. Follow your written appointment and medicine plan. A calendar does not authorise a dose.

Support around the medicines

1 **Blood counts and infection**

Fever, shivers or sudden illness needs prompt advice on any day.

2 **Nerve symptoms**

Report numbness, tingling or weakness before the next dose.

3 Support medicines

Follow the prescription for the active medicine and treatment day.

4 Review the plan

Discuss symptoms, recovery and the next treatment decision with the team.

Questions worth asking

Who arranges the checks and explains the results?

Agree the due window, booking team, result reviewer and contact route if a test or appointment is missing. This page does not record completed checks.

Try explaining the plan back: Which medicines and doses are prescribed, when are they due, which checks are needed, and what would make you call the team today?

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Phase 1 · dose-dense AC

Active medicines: Doxorubicin + Cyclophosphamide

Emetogenic risk: High · AC

Medicine	Dose / route	When
Aprepitant	125 mg orally	Day 1
Ondansetron	16 mg orally	Day 1 Individual dose / QT / hepatic review
Dexamethasone	12 mg orally	Day 1 No routine Days 2–4 dexamethasone in this NCCP AC antiemetic row
Olanzapine	5 mg orally	Days 1–4 Review sedation, falls, interactions and glucose
Aprepitant	80 mg orally daily	Days 2–3
Metoclopramide	10 mg orally up to three times daily	Subsequent days: duration and use to be confirmed Caution with olanzapine: additive extrapyramidal effects; adjust / avoid for individual contraindications

Additional phase-specific support

- granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) is required for every dose-dense AC cycle. Prescribe product, dose and timing separately under the approved local plan.
- This script applies only to AC. It is replaced when weekly paclitaxel starts.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Phase 2 · weekly paclitaxel

Active medicines: Paclitaxel

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally	On treatment day Review QT, constipation and hepatic dosing; reconcile any steroid already prescribed
Metoclopramide	10 mg orally up to three times daily as needed	Subsequent days: duration to be confirmed Assess contraindications, renal function and interactions

Additional phase-specific support

- First paclitaxel dose: dexamethasone 8 mg intravenous (IV), chlorphenamine 10 mg IV and famotidine 20 mg IV, each 30 minutes before treatment (NCCP suggested table).
- From Day 8 onward: chlorphenamine 10 mg IV and famotidine 20 mg IV 30 minutes before treatment. Routine dexamethasone may be omitted after the first dose; prior reaction or increased risk requires consultant review.
- Famotidine may be omitted under the consultant/local plan if appropriate. A prior reaction requires a separate rechallenge assessment; do not reuse the standard draft automatically.
- Growth-factor support is required during dose-dense AC; do not automatically continue it during weekly paclitaxel without clinical need.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Hair / scalp

Chemotherapy

Hair loss or thinning varies; ask about support.

2 Mouth

Dose-dense AC → weekly paclitaxel

Painful ulcers or difficulty drinking need advice.

3 Breathing

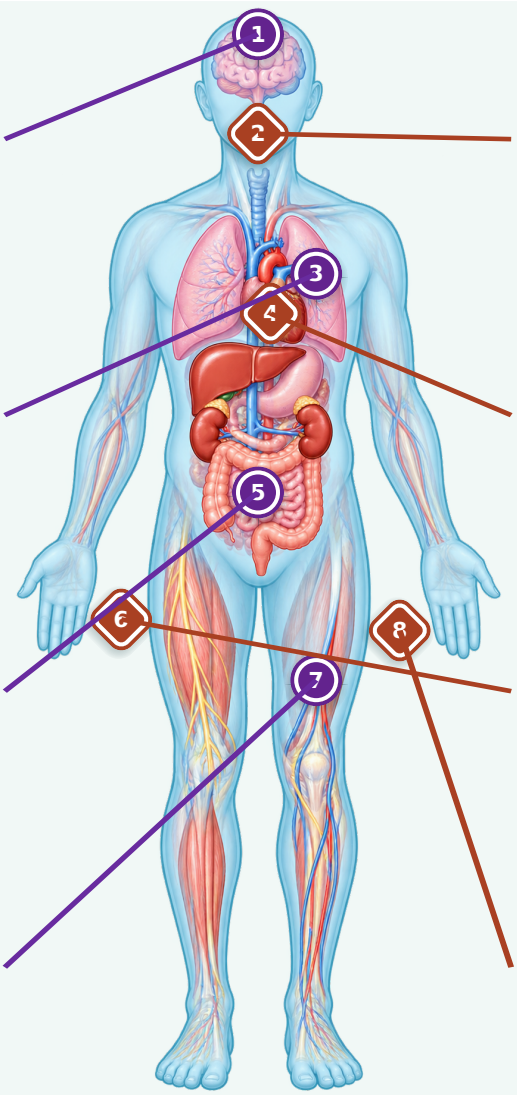
Dose-dense AC → weekly paclitaxel

New cough or breathlessness needs prompt assessment.

4 Heart

Dose-dense AC → weekly paclitaxel

New chest pain, fainting or severe breathlessness needs urgent assessment.



5 Bowel

Dose-dense AC → weekly paclitaxel

Report diarrhoea early, especially with poor intake or dizziness.

6 Nerves / joints

Dose-dense AC → weekly paclitaxel

Report numbness, tingling or pain affecting daily activity.

7 Blood / infection

Dose-dense AC → weekly paclitaxel

Fever, shivers, unusual bleeding or sudden illness: call promptly.

8 Skin / liver symptoms

Dose-dense AC → weekly paclitaxel

Report severe rash, blistering or yellow skin / eyes promptly.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Report new breathlessness, jaundice, marked bleeding, severe diarrhoea, inability to drink, chest symptoms or a severe rash promptly.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Your record and contact card

IN THIS CHAPTER

Space for your own cycle or review record, questions and notes, followed by your treatment card.

Record each cycle separately. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Use a new record sheet for each cycle or review. Copy or print another sheet when needed; the number of sheets does not prescribe the length of treatment.

Cycle number: _____ Start date: _____ Next review: _____

AC chemotherapy · reusable record · copy this sheet for another cycle of the same phase.

Doxorubicin · Anthracycline chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Mouth soreness / ulcers	□ None	□ Mild soreness; eating normally	□ Painful; soft food needed: call	□ Cannot eat / drink enough: urgent review	□ Major breathing difficulty: 112/999
Nausea	□ None	□ Queasy but eating and drinking	□ Intake reduced: contact the team	□ Unable to maintain intake: urgent review	□ Collapse / severe dehydration: 112/999
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999

Cyclophosphamide · Alkylating chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Nausea	□ None	□ Queasy but eating and drinking	□ Intake reduced: contact the team	□ Unable to maintain intake: urgent review	□ Collapse / severe dehydration: 112/999
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999
Urinary symptoms	□ Usual pattern	□ Mild change: report	□ Pain or frequent urination: call	□ Cannot pass urine / marked bleeding: urgent review	□ Collapse or severe illness: 112/999

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Cycle number: _____ Start date: _____ Next review: _____

Weekly paclitaxel · reusable record · copy this sheet for another cycle of the same phase.

Paclitaxel · Taxane chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Numbness / tingling	□ None	□ Mild change; usual tasks unaffected	□ Limits buttons, writing or walking: call	□ Washing, dressing or walking difficult: urgent review	□ Sudden major weakness / paralysis: emergency help
Muscle / joint pain	□ None	□ Mild aches; normal activity	□ Pain limits tasks / sleep: call	□ Pain limits self-care: urgent review	□ Sudden severe deterioration: emergency help
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999
Mouth soreness / ulcers	□ None	□ Mild soreness; eating normally	□ Painful; soft food needed: call	□ Cannot eat / drink enough: urgent review	□ Major breathing difficulty: 112/999
Diarrhoea	□ Usual pattern	□ Up to 3 extra stools above normal	□ 4–6 extra stools/ day or at night: call	□ 7+ extra stools/ day or dehydration: urgent review	□ Collapse / severe dehydration: 112/999
Nausea	□ None	□ Queasy but eating and drinking	□ Intake reduced: contact the team	□ Unable to maintain intake: urgent review	□ Collapse / severe dehydration: 112/999

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Keep your treatment details to hand

Adjuvant or neoadjuvant high-risk node-negative or node-positive breast cancer · NCCP 00485 v6 · Document revised 4 November 2026

Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

FRONT / CONTACTS

My Treatment Card

SACTCheck

Name: _____

AC then weekly paclitaxel

AC every 14 days ×4; then paclitaxel weekly ×12. Record the phase with each entry.

Phase: _____ Cycle plan: team to confirm

Daytime (EXAMPLE) 087 12345678
Out of hours (EXAMPLE) 087 87654321

Unwell or fever? Call oncology now.
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

CUT outer border

BACK / TREATMENT RECORD

My cycle record

Rolling record — follow your agreed treatment plan.

Cycle	Date / initials

My treatment information

Latest treatment date: _____

Plan changed? Ask the team to update this card.

FOLD centre — do not cut

Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

Your local contacts and resources

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Hospital / team: _____
Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

SACTCheck v0.84.32 · NCCP 00485 v6 · Wallet-card sheet
Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

CHAPTER 05

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00485 v6 · official regimen**
   https://healthservice.hse.ie/documents/6500/485_v6_Dose_DenseAC-T.pdf
2. **ESMO 2024 early breast cancer guideline · treatment and follow-up**
   https://www.annalsofoncology.org/article/S0923-7534(23)05104-9/fulltext
3. **NICE NG101 · bone health and follow-up**
   https://www.nice.org.uk/guidance/ng101/chapter/Recommendations
4. **eviQ · implanted port patient information**
   https://www.eviq.org.au/p/507
5. **eviQ · peripherally inserted central catheter (PICC) patient information**
   https://www.eviq.org.au/p/543
6. **ESC 2022 cardio-oncology guideline · cardiac surveillance and risk**
   https://academic.oup.com/eurheartj/article/43/41/4229/6673995
7. **HSE · chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
8. **NOGG · section 7, aromatase inhibitors and fracture risk**
   https://www.nogg.org.uk/full-guideline/section-7-strategies-management-osteoporosis-and-fracture-risk
9. **NCCP NCIS antiemetics v8 (2025), Table 1**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
10. **ASCO · biomarkers in metastatic breast cancer**
    https://ascopubs.org/doi/10.1200/JCO.2015.61.1459
11. **ESMO metastatic breast cancer guideline · 2026**
    https://www.annalsofoncology.org/article/S0923-7534(26)00902-6/fulltext